Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Tonsillectomy

Removal of the tonsils (complete or partial)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Planned technique:
☐ Cold-steel dissection ☐ Diathermy (electrocautery)
☐ Radiofrequency / coblation ☐ Tonsillotomy (partial removal)

Additional procedures in the same session:
☐ Adenoidectomy ☐ Ventilation tube(s) ☐ None _____

Reason for surgery (completed by the doctor):
☐ Recurrent tonsillitis ☐ Sleep-disordered breathing / snoring
☐ Peritonsillar abscess (quinsy)
☐ One-sided enlargement / tissue diagnosis _____

Other / comments: _____

1 Your condition and the aim of the procedure

The tonsils are lymphatic tissue, part of the body's immune system, located on both sides of the throat above the base of the tongue. After the tonsils are removed, the immune system continues to work through the other lymphatic tissues of the mouth and throat; the operation has not been shown to weaken immunity.

Tonsillectomy is most often recommended for: **tonsillitis** that recurs several times a year and causes time off school or work; enlarged tonsils causing **snoring and pauses in breathing during sleep**; an **abscess around the tonsil (quinsy)**; or one-sided enlargement of a tonsil or a suspicious change in the tissue. In some patients, bad breath and tonsil stones may also be reasons.

The aim of the operation is to eliminate or reduce these problems. Your doctor will explain separately why the operation has been recommended for you or your child.

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2 How is the procedure performed?

The operation is performed under general anaesthesia in an operating theatre, through the mouth; **there are no cuts or scars on the outside.**

1. **Preparation:** After the anaesthetic has been given, a special instrument is placed to keep the mouth open and protect the tongue.
2. **Removing the tonsils:** Each tonsil is separated, together with its surrounding capsule, from the muscle layer beneath and removed. Depending on the planned technique this is done with scissors and dissecting instruments (cold-steel dissection), electrocautery or radiofrequency/coblation devices.
3. **Partial removal (tonsillotomy):** In suitable patients, especially young children with breathing problems during sleep, only the enlarged part of the tonsil may be removed, leaving a thin layer behind. This method may cause less pain and a lower risk of bleeding; however, the remaining tissue can regrow or become infected.
4. **Controlling bleeding:** Any bleeding in the tonsil beds is carefully stopped with stitches, ties or cautery. The tonsil beds are usually left open and heal by themselves.
5. **Additional procedures:** If planned, removal of the adenoids (adenoidectomy) or insertion of ventilation tubes (grommets) is carried out under the same anaesthetic; separate consent forms are completed for these procedures.
6. **Waking up and observation:** After the operation the patient is monitored in the recovery room and then on the ward for bleeding, breathing and fluid intake.

If the doctor considers it necessary, the removed tonsils are sent for pathological examination.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 30–45 minutes (excluding anaesthesia time)	Usually one night in hospital for observation

3 Expected benefits and likelihood of success

- A marked reduction in recurrent tonsillitis and the associated fever, sore throat, antibiotic use and time off school or work
- Less snoring and fewer pauses in breathing during sleep; better-quality sleep and improved daytime attention and behaviour
- Prevention of recurrent peritonsillar abscess
- Where needed, a definite diagnosis through pathological examination of the tissue

In carefully selected patients, recurrent throat infections and sleep-disordered breathing improve to a large extent. Occasional throat infections (pharyngitis) can still occur after the operation, but they are generally less frequent and milder. If sleep apnoea has other causes as well — such as excess weight, facial structure or nasal blockage — symptoms may not resolve completely and further treatment may be needed.

4 Other treatment options

- **Watchful waiting:** If the frequency of infections does not meet the criteria for surgery or symptoms are mild, waiting with regular check-ups may be appropriate. In children, tonsillitis often becomes less frequent with age.
- **Antibiotics and supportive treatment:** Each episode of infection continues to be treated with medicines; this does not prevent further episodes.
- **Partial removal (tonsillotomy):** May be considered instead of complete removal in suitable patients (explained above).
- **Other options for sleep apnoea:** Weight loss, treatment of nasal blockage, steroid nasal sprays or a positive airway pressure device (CPAP) during sleep.

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5 What happens if the procedure is not performed?

Without surgery, recurrent tonsillitis, fever, antibiotic use and time off school or work may continue. Rarely, an abscess may develop around the tonsil or infection may spread into the tissues of the neck. If sleep apnoea due to enlarged tonsils is left untreated, it can — especially in children — lead to slower growth and development, attention and behaviour problems, bedwetting and, in the long term, strain on the heart and lungs. In suspicious situations such as one-sided enlargement, not operating may delay a definite diagnosis.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- Blood tests (particularly bleeding and clotting tests) and any other necessary investigations are carried out before the operation, and you will be assessed by the anaesthetist.
- Please tell us if you or anyone in your family has a tendency to bleed easily, frequent nosebleeds, heavy or prolonged periods, bleeding that would not stop after a tooth extraction, or a known bleeding disorder.
- No solid food or milk should be taken for **6 hours** before the operation; for clear fluids, follow the time given by the anaesthetist.
- Please let us know about any active tonsillitis, feverish illness or flu in the 2–3 weeks before the operation; it may be necessary to postpone it.
- Tell your doctor about any loose teeth or dentures.
- If you are travelling from abroad, we recommend staying within reach of your doctor and hospital for **at least 10–14 days** after the operation (because of the risk of delayed bleeding).
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

Day of surgery	You will have a sore throat when you wake up; this is controlled with painkillers. Once any nausea has passed, water is given, then cool or lukewarm drinks and soft foods. You will stay in hospital overnight to be monitored for bleeding, breathing and fluid intake.
Days 1–4	Throat pain and pain spreading to the ears are noticeable. A **white-yellowish layer** forms on the tonsil beds; this is a normal part of healing, not an infection. Bad breath and a slight temperature may occur. **Taking painkillers regularly and drinking plenty of fluids** are the most important things.
Days 5–10	The white layer begins to come away. Pain can sometimes increase again during this period. **This is when the risk of delayed bleeding is highest**; avoid strenuous exercise, hot baths/saunas and hard foods, and seek help immediately if there are any signs of bleeding.
Days 10–14	Pain decreases noticeably and a normal diet is resumed. Most children return to school, and adults to work, within 10–14 days. Sport and heavy physical activity are generally allowed after about 2–3 weeks.

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Weeks 3–6	The tonsil beds heal completely. A follow-up examination is carried out; if a pathological examination was done, the result is reviewed.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Sore throat (usually lasts 7–14 days; more marked in adults than in children)
- Pain spreading to the ears (referred pain; it does not mean there is a problem in the ear)
- Difficulty swallowing and loss of appetite; some weight loss in the first few days
- White-yellow healing layer on the tonsil beds and bad breath
- Slight temperature (first few days), nausea and vomiting from the anaesthetic
- Temporary swelling of the tongue and uvula

Common conditions (most are temporary or resolve with treatment)

- **Bleeding after the operation (delayed bleeding)** — Occurs in about **2–5 in 100** patients, most often **5–10 days after surgery**, when the healing layer comes away. It is somewhat more common in adults than in children. Most bleeds are minor and stop with observation, ice, treatment inside the mouth or a simple intervention.
- **Return to theatre for bleeding** — In about **1–3 in 100** patients, a **return to the operating theatre under general anaesthesia** may be needed to stop the bleeding. The hospital stay may be longer.
- **Not drinking enough and dehydration** — If not enough fluid is taken because of pain, weakness and reduced urine may occur, especially in young children; sometimes admission to hospital for intravenous fluids is needed.

Uncommon conditions

- **Blood transfusion** — If a lot of blood is lost during a bleed, a blood transfusion may be needed; this happens in **far fewer than 1 in 100** patients.
- **Taste disturbance and numbness of the tongue** — Because the tongue is held under pressure by the instrument during the operation, there may be a change in taste or numbness of the tongue; this usually improves within a few weeks, but rarely may last for months or be permanent.
- **Injury to the teeth, lips and tongue** — Pressure from the instrument used to keep the mouth open may chip a tooth (particularly a loose or decayed one), or cause a small burn, sore or swelling on the lips or tongue.
- **Infection** — Infection may develop in the tonsil beds or in the neck; it is usually treated with antibiotics.
- **Temporary nasal-sounding voice** — Because movement of the palate is limited by pain, air may temporarily escape through the nose when speaking; this usually settles within a few weeks.
- **Regrowth after tonsillotomy** — If partial removal was performed, the remaining tonsil tissue may regrow or become infected (about **1–6 in 100**); rarely, a complete tonsillectomy may then be needed.

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Rare or very rare, but potentially serious conditions

- **Severe bleeding and its consequences** — Rarely, bleeding may be heavy and rapid. In this case **emergency surgery, blood transfusion** and, very rarely, additional procedures on the blood vessels of the neck may be needed. Heavy blood loss or blood entering the airway can lead to serious consequences such as **breathing failure, the need for treatment in intensive care** and resulting permanent harm.
- **Life-threatening complications and death** — Death related to tonsillectomy is extremely rare; it is reported in the medical literature as about **1 in 15,000–35,000 operations**. The most common causes are severe bleeding and breathing problems. Young children with severe sleep apnoea have a higher risk of breathing problems after surgery, so these patients are monitored more closely.
- **Permanent nasal speech / fluids coming back through the nose (velopharyngeal insufficiency)** — Very rare; seen mainly in those with a hidden cleft of the palate. Speech therapy or, rarely, a corrective operation may be needed.
- **Slipping of the neck vertebrae (Grisel syndrome), infection spreading into the neck or chest** — Very rare conditions that can be treated if recognised early. They may show as steadily worsening neck pain, inability to turn the head, high fever and inability to swallow. If they occur, hospital admission, intravenous antibiotics and, if necessary, surgical drainage are needed; if treatment is delayed, lasting neck problems may develop or a spreading infection can become life-threatening.
- **Scarring and narrowing of the throat and palate** — Very rarely, excessive scar tissue may form during healing, narrowing the throat or palate and causing difficulty swallowing or speaking.
- **Jaw joint pain or dislocation** — Very rarely occurs as a result of the mouth being held open for a long time.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

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9 When to contact us immediately

- **Fresh, red blood** from the mouth, spitting or vomiting blood — **even a few drops** — seek help without delay. If bleeding occurs, lean your head forward and spit the blood out rather than swallowing it; you may gargle with cold water.
- In children: **frequent swallowing**, paleness, weakness, difficulty waking (may be a sign of swallowed blood)
- **Difficulty breathing**, marked pauses in breathing during sleep, noisy breathing
- **Fever above 38.5 °C**, or pain that does not settle with painkillers or keeps getting worse
- Not drinking at all, no urine for 8 hours, unusual drowsiness (signs of dehydration)
- Steadily worsening neck pain, inability to turn the head, swelling in the neck

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- For pain, use only **the painkillers recommended by your doctor**, at the recommended dose and intervals, and **take them regularly**. Aspirin and other painkillers not recommended by your doctor may increase the risk of bleeding; do not add any medicine on your own.
- **Drinking plenty of fluids** is the most important rule. Cool or lukewarm water, ayran (yoghurt drink), stewed fruit, ice cream, milk pudding, yoghurt and purées are suitable. Avoid very hot, acidic, spicy and hard/crunchy foods (crisps, crackers, toast) for about **2 weeks**.
- For the first 2 weeks, avoid strenuous exercise, hot baths, saunas and long periods in the sun.
- You may continue to brush your teeth; avoid any hard contact with the throat.
- We recommend postponing your return flight abroad until the risk of delayed bleeding has passed (usually **10–14 days after surgery**). If you need to fly earlier, discuss the risks with your doctor.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

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12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.

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☐ It has been clearly explained to me that **delayed bleeding may occur** within the first 2 weeks after surgery, especially on days 5–10; that in that case I must contact the doctor or go to the nearest emergency department without delay; and that bleeding may require a further operation, a blood transfusion and, rarely, treatment in intensive care.

☐ **I GIVE MY CONSENT to the procedure described above.**

☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

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This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.